allowed the WHO team on ground yet.[7] The truth, at that point, was only what the Chinese authorities chose to tell the WHO. China had been a menacing source of some of the most virulent outbreaks: SARS (severe acute respiratory syndrome), H5N1 and H7N9 bird flu. If the new virus was anything like the SARS outbreak of 2002, which started in a bat and transferred to a civet cat sold at the wet market, it would be some solace. SARS wasn't particularly infectious until the fifth or sixth day of the disease onset, by which time the patients would develop severe symptoms. This pattern was considered to have limited human-to-human transmission because people who displayed illness could easily be isolated, thus preventing others from being infected. Having said that, it did claim over 800 lives worldwide. Although India seemed more or less insulated from it, we had put systems in place like temperature screenings at airports, created designated hospitals with isolation wards and ensured effective communication to incoming passengers to watch out for symptoms and report to local health authorities in case of illness. We had also expanded a five-year pilot project undertaken by the National Surveillance Programme for Communicable Diseases (NSPCD) from five to a hundred districts to track, trace and treat. Back then, the former director general of health services in the Ministry of Health and Family Welfare (MoHFW), Dr S.P. Agarwal, noted, 'The SARS coronavirus is neither the first nor the last virus to emerge as a major public health threat. The SARS epidemic should therefore be considered as an opportunity to improve public health systems.' In fact, it was precisely actions from government memory at the time of SARS which greatly contributed to initiate quick strategies when the novel coronavirus (nCov)-19 outbreak hit us. However, if the new virus was anything like the H5N1 in 2006 and H7N9 in 2014 which had claimed the lives of the 60 per cent and 40 per cent of those infected, respectively, it would be extremely worrying.

The PMO and their trusted experts for the job, NITI Aayog[8] member Dr V.K. Paul and Principal Scientific Advisor (PSA) Prof. K. VijayRaghavan got to work. They compiled regular reports on the updated information which was emerging from China. Prof. VijayRaghavan was a distinguished biologist and had served as secretary of the Department of Biotechnology and chairman of the board of Coalition for Epidemic Preparedness Innovations (CEPI). Over the years, Prof. VijayRaghavan had been deeply involved in the study of various disease-causing agents and had been